NCCP 00849 · clinician draft

Nivolumab, paclitaxel and carboplatin before surgery

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · paclitaxel / carboplatin with nivolumab

Active medicines: nivolumab + paclitaxel + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Each Day 1 chemotherapy visit
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Each Day 1 chemotherapy visit The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.
Paclitaxel dexamethasone premedication	Source reference 20 mg oral or IV	Oral approximately 6 and 12 hours before paclitaxel, or IV 30 minutes before Reconcile with antiemetic steroid; source advises considering 12 mg on treatment day when aprepitant is added. Consultant may reduce/omit only under explicit guidance.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Omission requires consultant guidance. Source cautions particularly with every-three-week paclitaxel; monitor closely for hypersensitivity.

Additional phase-specific support

- Take the prescribed paclitaxel allergy-prevention medicines; your team reconciles steroids with anti-sickness treatment. Carboplatin dosing uses a verified kidney-function measurement; report dehydration or a change in health.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.
- The prescriber specifies doses, duration, rescue treatment and interactions; follow one reconciled prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00849 v2 · pNCCP 00849 pp1–7; NCCP 00304 supportive care; NCIS V8 Table 1 · NCCP NCIS antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required